

## Analgesic Tapering Guidance

### Strong opioids and gabapentinoids prescribed for chronic pain in adults

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| Chronic pain is difficult to treat with medication, and in particular there is little evidence that opioids are helpful.                                                                                                                                                                                                                                                                                                                           |
| The risk of harm increases at doses above 120mg oral morphine equivalent (OME) daily. Above this dose the risk of harm and mortality increases substantially, but there is no increased benefit ( <a href="#">Opioids Aware dose equivalents</a> ).                                                                                                                                                                                                |
| Medication should only be continued when a person is demonstrating pain reduction and functional improvement.                                                                                                                                                                                                                                                                                                                                      |
| When adverse effects or lack of pain reduction and/or lack of functional improvement are noted, it is safest to reduce (and where possible stop) the medication, even if there is no alternative medication treatment option.                                                                                                                                                                                                                      |
| Report of lessening effect even with increasing dose is a potential sign of treatment failure. Evidence suggests reducing medication to lowest tolerated dose / a stop, rather than continuing or increasing doses further.                                                                                                                                                                                                                        |
| A small proportion of people may obtain good pain relief with opioids in the long term if the dose can be kept low and use is intermittent.                                                                                                                                                                                                                                                                                                        |
| The aim of management of chronic pain is empowerment of people to self-manage pain and increase function using safer and more effective methods. <i>Non-pharmacological methods of managing chronic pain include increasing activity and physical fitness, physiotherapy, hot or cold pack application, transcutaneous electrical nerve stimulation (TENS), cognitive behavioural therapy (CBT) and meditation techniques such as mindfulness.</i> |
| A detailed assessment of the emotional influences on the person's pain experience is essential for people with chronic pain who also have refractory and disabling symptoms, particularly if they are prescribed high opioid doses. <i>These people may need specialist support.</i>                                                                                                                                                               |
| Dose reductions should be agreed with the person wherever possible, with explanation of the benefits of stopping an opioid. <i>However, lack of engagement is not a reason to avoid gradually tapering medication when there are safety concerns or it is believed to be in the best interest of the person.</i>                                                                                                                                   |
| Total daily opioid dose should be reduced gradually when a person has been prescribed a strong opioid for longer than two weeks. <i>The total daily dose can be reduced by 10% of the original dose weekly or every two weeks, but it is generally accepted this may be too rapid when dose reductions are undertaken in Primary Care.</i>                                                                                                         |
| Reduce one medication at a time, deciding with the person which medication they would like to reduce first. Doses $\geq 120$ mg OME daily should be highlighted as a priority. <i>Take into account 'when required' doses of opioid medication on an individual basis, ensuring dose is not escalated while reduction of regular analgesic medication occurs.</i>                                                                                  |
| Make small dose changes, which are less likely to cause withdrawal symptoms and will build confidence in the tapering process. <i>Where possible engage the person in deciding which dose they would like to reduce initially, i.e. morning / evening dose.</i>                                                                                                                                                                                    |
| Dose reductions can be made weekly (except in the instance of transdermal fentanyl), two weekly or monthly, and should be guided by what the person can manage. <i>Frequency of dose reduction can change during the tapering process.</i>                                                                                                                                                                                                         |
| Document the dose reduction plan clearly for the person reducing their opioid or gabapentinoid medication, using the template available on <a href="#">East Sussex Formulary</a> .                                                                                                                                                                                                                                                                 |
| When reducing prescribe exact quantities, clearly document the dose reduction on the medication dosing instructions, remove the medication being reduced from repeat prescription, and provide enough medication for a maximum of two reductions at a time.                                                                                                                                                                                        |

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| People reducing their opioid or gabapentinoid dose should be reviewed at least every two weeks, with assessment of pain, level of function, and signs of withdrawal. <i>The initial consultation should be face-to-face to explain why the opioid is being reduced, but it may be possible for subsequent consultations to be conducted via telephone.</i> |
| Clinicians are encouraged to signpost people with chronic pain to <a href="#">My Live Well with Pain</a> . The site contains trusted techniques people with chronic pain have found useful in helping them improve their quality of life and live well with their pain.                                                                                    |
| Tapering opioid and gabapentinoid medication doses safely is potentially time-consuming, but is extremely worthwhile. People generally report quality of life improvements with medication tapering in chronic pain.                                                                                                                                       |

### Strong opioid suggested tapering regimes

Opioids are good analgesics when used for acute pain and for pain at the end of life but there is little evidence they are helpful for chronic pain. The side effects and long-term adverse outcomes of opioids are well documented, with the risk of harm of opioid prescribing increasing substantially at doses above an oral morphine equivalent (OME) of 120mg/day and no additional benefit demonstrated. Long-term risks of opioid prescribing include dependence, opioid induced hyperalgesia, endocrine abnormalities, cognitive decline and falls.

Suggested tapering regimes are for guidance and are not prescriptive; each taper needs to be individualised to the person making the reductions. The aim is not necessarily to stop the opioid medication, but there should be efforts made to reduce risk with a particular focus on reduction to safer levels of prescribing in instances where people are prescribed an OME  $\geq 120$ mg/day for chronic pain. Agree a maximum of two reductions at each review.

#### **Morphine modified release (MR) e.g. starting at 200mg MR twice daily**

*Dose changes should be individualised to the person, and made not more frequently than weekly.*

| Change<br>(e.g. weekly / fortnightly / monthly) | Morning morphine MR dose | Evening morphine MR dose |
|-------------------------------------------------|--------------------------|--------------------------|
| 1                                               | 190mg                    | 190mg*                   |
| 2                                               | 180mg                    | 180mg*                   |
| 3                                               | 170mg                    | 170mg*                   |
| 4                                               | 160mg                    | 160mg*                   |
| 5                                               | 150mg                    | 150mg*                   |
| 6                                               | 140mg                    | 140mg*                   |
| 7                                               | 130mg                    | 130mg*                   |
| 8                                               | 120mg                    | 120mg*                   |
| 9                                               | 110mg                    | 110mg*                   |
| 10                                              | 100mg                    | 100mg*                   |
| 11                                              | 90mg                     | 100mg                    |
| 12                                              | 90mg                     | 90mg                     |
| 13                                              | 80mg                     | 90mg                     |
| 14                                              | 80mg                     | 80mg                     |
| 15                                              | 70mg                     | 80mg                     |

|    |        |        |
|----|--------|--------|
| 16 | 70mg   | 70mg   |
| 17 | 60mg   | 70mg   |
| 18 | 60mg   | 60mg   |
| 19 | 50mg   | 60mg   |
| 20 | 50mg   | 50mg   |
| 21 | 45mg** | 50mg   |
| 22 | 45mg** | 45mg** |
| 23 | 40mg   | 45mg** |
| 24 | 40mg   | 40mg   |
| 25 | 35mg** | 40mg   |
| 26 | 35mg** | 35mg** |
| 27 | 30mg   | 35mg** |
| 28 | 30mg   | 30mg   |
| 29 | 25mg** | 30mg   |
| 30 | 25mg** | 25mg** |
| 31 | 20mg   | 25mg** |
| 32 | 20mg   | 20mg   |
| 33 | 15mg** | 20mg   |
| 34 | 15mg** | 15mg** |
| 35 | 10mg   | 15mg** |
| 36 | 10mg   | 10mg   |
| 37 | 5mg**  | 10mg   |
| 38 | 5mg**  | 5mg**  |
| 49 | STOP   | 5mg**  |
| 40 | STOP   | STOP   |

\*Higher dose can be 'see-sawed' down, reducing only one of the doses at a time, if changing both doses simultaneously is too much for someone.

\*\*Total dose will require the use of 5mg modified release morphine tablets (currently only available as MST Continus®).

#### **Oxycodone modified release (MR) e.g. starting at 100mg MR twice daily**

Dose changes should be individualised to the person, and made not more frequently than weekly.

| Change<br>(e.g. weekly / fortnightly / monthly) | Morning oxycodone MR dose | Evening oxycodone MR dose |
|-------------------------------------------------|---------------------------|---------------------------|
| 1                                               | 95mg                      | 95mg*                     |
| 2                                               | 90mg                      | 90mg*                     |
| 3                                               | 85mg                      | 85mg*                     |
| 4                                               | 80mg                      | 80mg*                     |
| 5                                               | 75mg                      | 75mg*                     |
| 6                                               | 70mg                      | 70mg*                     |
| 7                                               | 65mg                      | 65mg*                     |
| 8                                               | 60mg                      | 60mg*                     |
| 9                                               | 55mg                      | 55mg*                     |
| 10                                              | 50mg                      | 50mg*                     |
| 11                                              | 45mg                      | 45mg*                     |

|    |      |       |
|----|------|-------|
| 12 | 40mg | 40mg* |
| 13 | 35mg | 40mg  |
| 14 | 35mg | 35mg  |
| 15 | 30mg | 35mg  |
| 16 | 30mg | 30mg  |
| 17 | 25mg | 30mg  |
| 18 | 25mg | 25mg  |
| 19 | 20mg | 25mg  |
| 20 | 20mg | 20mg  |
| 21 | 15mg | 20mg  |
| 22 | 15mg | 15mg  |
| 23 | 10mg | 15mg  |
| 24 | 10mg | 10mg  |
| 25 | 5mg  | 10mg  |
| 26 | 5mg  | 5mg   |
| 27 | STOP | 5mg   |
| 28 | STOP | STOP  |

\*Higher dose can be 'see-sawed' down, reducing only one of the doses at a time, if changing both doses simultaneously is too much for someone

**Tapentadol modified release (MR) e.g. starting at 250mg twice daily**

Dose changes should be individualised to the person, and made not more frequently than weekly.

| Change<br>(e.g. weekly / fortnightly / monthly) | Morning tapentadol MR dose | Evening tapentadol MR dose |
|-------------------------------------------------|----------------------------|----------------------------|
| 1                                               | 200mg                      | 250mg                      |
| 2                                               | 200mg                      | 200mg                      |
| 3                                               | 150mg                      | 200mg                      |
| 4                                               | 150mg                      | 150mg                      |
| 5                                               | 100mg                      | 150mg                      |
| 6                                               | 100mg                      | 100mg                      |
| 7                                               | 50mg                       | 100mg                      |
| 8                                               | 50mg                       | 50mg                       |
| 9                                               | STOP                       | 50mg                       |
| 10                                              | STOP                       | STOP                       |

Tapentadol can be difficult to reduce, due to its dual action as an opioid and noradrenaline reuptake inhibitor (making its effects less predictable). Tapentadol 50mg is approximately equivalent to oral morphine 20mg, meaning removing morning and evening doses simultaneously is generally too fast. *It is suggested doses are 'see-sawed' down.* In the instance a someone finds reducing tapentadol too difficult, they could be considered (if appropriate) for conversion to the oral morphine equivalent dose to enable more gradual tapering.

**Transdermal fentanyl e.g. starting at 200 micrograms/hour (changed every three days)**

*It is suggested that dose changes are not made more frequently than every two weeks, given transdermal fentanyl 12 micrograms/hours is approximately equivalent to 45mg oral morphine.*

| Change<br>(e.g. fortnightly / monthly) | Transdermal fentanyl dose | How to prescribe                |
|----------------------------------------|---------------------------|---------------------------------|
| 1                                      | 187 micrograms/hour       | 100mcg/hr + 75mcg/hr + 12mcg/hr |
| 2                                      | 175 micrograms/hour       | 100mcg/hr + 75mcg/hr            |
| 3                                      | 162 micrograms/hour       | 100mcg/hr + 50mcg/hr + 12mcg/hr |
| 4                                      | 150 micrograms/hour       | 100mcg/hr + 50mcg/hr            |
| 5                                      | 137 micrograms/hour       | 100mcg/hr + 25mcg/hr + 12mcg/hr |
| 6                                      | 125 micrograms/hour       | 100mcg/hr + 25mcg/hr            |
| 7                                      | 112 micrograms/hour       | 100mcg/hr + 12mcg/hr            |
| 8                                      | 100 micrograms/hour       | 100mcg/hr                       |
| 9                                      | 87 micrograms/hour        | 75mcg/hr + 12mcg/hr             |
| 10                                     | 75 micrograms/hour        | 75mcg/hr                        |
| 11                                     | 62 micrograms/hour        | 50mcg/hr + 12mcg/hr             |
| 12                                     | 50 micrograms/hour        | 50mcg/hr                        |
| 13                                     | 37 micrograms/hour        | 25mcg/hr + 12mcg/hr             |
| 14                                     | 25 micrograms/hour*       | 25mcg/hr                        |
| 15                                     | 12 micrograms/hour*       | 12mcg/hr                        |
| 16                                     | STOP                      | STOP                            |

\*At lower transdermal fentanyl doses it might be easier for the person to be converted to the oral morphine equivalent dose (if appropriate to do so), for more gradual tapering.

#### Transdermal buprenorphine e.g. starting at 140 micrograms/hour

Example uses preparations that should be applied every 96 hours for higher doses, and preparations that should be changed weekly at lower doses.

| Change<br>(e.g. weekly / fortnightly / monthly) | Transdermal buprenorphine dose | How to prescribe                               | Available preparations                                                                                                                                 |
|-------------------------------------------------|--------------------------------|------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------|
| 1                                               | 122.5 micrograms/hour          | 1x70mcg/hr + 1x52.5mcg/hr changed every 96hrs* | Bupeaze®,<br>Buplast®,<br>Relevtec®,<br>Transtec®                                                                                                      |
| 2                                               | 105 micrograms/hour            | 1x70mcg/hr + 1x35mcg/hr changed every 96hrs*   |                                                                                                                                                        |
| 3                                               | 87.5 micrograms/hour           | 1x52.5mcg/hr + 1x35mcg/hr changed every 96hrs* |                                                                                                                                                        |
| 4                                               | 70 micrograms/hour             | 1x70mcg/hr changed every 96hrs*                |                                                                                                                                                        |
| 5                                               | 52.5 micrograms/hour           | 1x52.5mcg/hr changed every 96hrs*              |                                                                                                                                                        |
| 6                                               | 40 micrograms/hour             | 2x20mcg/hr changed weekly**                    | Bunov®,<br>Bupramyl®, Butec®,<br>BuTrans®, Panitaz®,<br>Reletrans®,<br>Sevodyne®<br>(15mcg/hr only<br>available as<br>Butec®, BuTrans®,<br>Reletrans®) |
| 7                                               | 35 micrograms/hour             | 1x20mcg/hr + 1x15mcg/hr changed weekly**       |                                                                                                                                                        |
| 8                                               | 30 micrograms/hour             | 1x20mcg/hr + 1x10mcg/hr changed weekly**       |                                                                                                                                                        |
| 9                                               | 25 micrograms/hour             | 1x20mcg/hr + 1x5mcg/hr changed weekly**        |                                                                                                                                                        |
| 10                                              | 20 micrograms/hour             | 1x20mcg/hr changed weekly**                    |                                                                                                                                                        |
| 11                                              | 15 micrograms/hour             | 1x15mcg/hr changed weekly**                    |                                                                                                                                                        |
| 12                                              | 10 micrograms/hour             | 1x10mcg/hr changed weekly**                    |                                                                                                                                                        |
| 13                                              | 5 micrograms/hour              | 1x5mcg/hr changed weekly**                     |                                                                                                                                                        |
| 14                                              | STOP                           | STOP                                           |                                                                                                                                                        |

\* Preparations that should be applied every 96 hours include Bupeaze®, Buplast®, Relevtec®, and Transtec®.

The suggested reduction regime cannot be used for preparations that should be applied every 72 hours, which include Hapoctasin® and Prenotrix®; seek additional guidance in the instance a person is prescribed a

*72 hour preparation, or in the instance a patch intended for 96 hour application is being change more frequently.*

*\*\*Preparations that should be applied weekly include Bupramyl®, Butec®, BuTrans®, Panitaz®, Reletrans®, and Sevodyne®; seek additional guidance in the instance a patch intended for weekly application is being changed more frequently.*

### **Gabapentinoid suggested tapering regimes**

Pregabalin and gabapentin should only be prescribed where there is evidence of neuropathic changes / neuropathic pain, and even then 50% of patients will not get any benefit at all. Prescribing of gabapentinoids for neuropathic pain should be reviewed in line with the criteria set out in [NICE CG173](#) for neuropathic pain and discontinued (gradually) if ineffective. Even people who think they might obtain benefit from the use of a pregabalin or gabapentin should undertake a trial dose reduction periodically, to ensure they are benefiting / to see if they get the same benefit on a lower dose. Gabapentinoids are very unlikely to be of benefit when prescribed for non-neuropathic pain.

Side effects include sedation, weight gain, mood changes, hallucinations, muscle and joint pain, sexual dysfunction, and impaired immune response. *Co-prescribing of opioids and gabapentinoids increases the risk of respiratory depression and accidental overdose, and should be avoided if possible.*

Suggested tapering regimes are for guidance and are not prescriptive; each taper needs to be individualised to the person making the reductions. The aim is not necessarily to stop the gabapentinoid medication, but there should be efforts made to reduce risk. Agree a maximum of two reductions at each review.

**Gabapentin e.g. starting at 1200mg three times a day (300mg decrease per dose reduction, with variable daily doses)**

*Dose changes should be individualised to the person, and made not more frequently than weekly.*

| <b>Change</b><br>(e.g. weekly / fortnightly / monthly) | <b>Morning gabapentin dose</b> | <b>Midday gabapentin dose</b> | <b>Evening gabapentin dose</b> |
|--------------------------------------------------------|--------------------------------|-------------------------------|--------------------------------|
| 1                                                      | 900mg                          | 1200mg                        | 1200mg                         |
| 2                                                      | 900mg                          | 900mg                         | 1200mg                         |
| 3                                                      | 900mg                          | 900mg                         | 900mg                          |
| 4                                                      | 600mg                          | 900mg                         | 900mg                          |
| 5                                                      | 600mg                          | 600mg                         | 900mg                          |
| 6                                                      | 600mg                          | 600mg                         | 600mg                          |
| 7                                                      | 300mg                          | 600mg                         | 600mg                          |
| 8                                                      | 300mg                          | 300mg                         | 600mg                          |
| 9                                                      | 300mg                          | 300mg                         | 300mg                          |
| 10                                                     | STOP                           | 300mg                         | 300mg                          |
| 11                                                     | STOP                           | STOP                          | 300mg                          |
| 12                                                     | STOP                           | STOP                          | STOP                           |

**Gabapentin e.g. starting at 1200mg three times a day (300mg decrease per dose reduction, with same reduction of 100mg to each of the daily doses)**

*Dose changes should be individualised to the person, and made not more frequently than weekly.*

| Change<br>(e.g. weekly / fortnightly / monthly) | Morning gabapentin dose | Midday gabapentin dose | Evening gabapentin dose |
|-------------------------------------------------|-------------------------|------------------------|-------------------------|
| 1                                               | 1100mg                  | 1100mg                 | 1100mg                  |
| 2                                               | 1000mg                  | 1000mg                 | 1000mg                  |
| 3                                               | 900mg                   | 900mg                  | 900mg                   |
| 4                                               | 800mg                   | 800mg                  | 800mg                   |
| 5                                               | 700mg                   | 700mg                  | 700mg                   |
| 6                                               | 600mg                   | 600mg                  | 600mg                   |
| 7                                               | 500mg                   | 500mg                  | 500mg                   |
| 8                                               | 400mg                   | 400mg                  | 400mg                   |
| 9                                               | 300mg                   | 300mg                  | 300mg                   |
| 10                                              | 200mg                   | 200mg                  | 200mg                   |
| 11                                              | 100mg                   | 100mg                  | 100mg                   |
| 12                                              | STOP                    | STOP                   | STOP                    |

In the instance someone is struggling with gabapentin dose reductions as suggested above, then consider reducing the dose more slowly, e.g. by 100mg of a single dose every week or every two weeks.

**Pregabalin e.g. starting at 300mg twice daily (50mg decrease per dose reduction, with variable daily doses)**

*Dose changes should be individualised to the person, and made not more frequently than weekly.*

| Change<br>(e.g. weekly / fortnightly / monthly) | Morning pregabalin dose | Evening pregabalin dose |
|-------------------------------------------------|-------------------------|-------------------------|
| 1                                               | 250mg                   | 300mg                   |
| 2                                               | 250mg                   | 250mg                   |
| 3                                               | 200mg                   | 250mg                   |
| 4                                               | 200mg                   | 200mg                   |
| 5                                               | 150mg                   | 200mg                   |
| 6                                               | 150mg                   | 150mg                   |
| 7                                               | 100mg                   | 150mg                   |
| 8                                               | 100mg                   | 100mg                   |
| 9                                               | 50mg                    | 100mg                   |
| 10                                              | 50mg                    | 50mg                    |
| 11                                              | STOP                    | 50mg                    |
| 12                                              | STOP                    | STOP                    |

**Pregabalin e.g. starting at 300mg twice daily (50mg decrease per dose reduction, with the same reduction of 25mg to each dose)**

*Dose changes should be individualised to the person, and made not more frequently than weekly.*

| Change<br>(e.g. weekly / fortnightly / monthly) | Morning pregabalin dose | Evening pregabalin dose |
|-------------------------------------------------|-------------------------|-------------------------|
| 1                                               | 275mg                   | 275mg                   |
| 2                                               | 250mg                   | 250mg                   |
| 3                                               | 225mg                   | 225mg                   |
| 4                                               | 200mg                   | 200mg                   |
| 5                                               | 175mg                   | 175mg                   |
| 6                                               | 150mg                   | 150mg                   |
| 7                                               | 125mg                   | 125mg                   |
| 8                                               | 100mg                   | 100mg                   |
| 9                                               | 75mg                    | 75mg                    |
| 10                                              | 50mg                    | 50mg                    |
| 11                                              | 25mg                    | 25mg                    |
| 12                                              | STOP                    | STOP                    |

In the instance someone is struggling with pregabalin dose reductions as suggested, then consider reducing the dose more slowly, e.g. by 25mg of a single dose every week or every two weeks.

### References and further supporting guidance

- [Opioids Aware](#) from the Faculty of Pain Medicine provides a wealth of information for people living with chronic pain and healthcare professionals on opioid medicines.
- [Oxford University Hospitals 'Resources for GPs regarding chronic pain'](#) has a suite of excellent resources to support tapering, including a dose equivalent calculator and auto-populating tapering for strong opioids.
- [Live Well with Pain](#), and its sister site for people with chronic pain [My Live Well with Pain](#), provide many practical resources, including videos, to support self-empowerment to manage pain through non-pharmacological methods.
- 'What interventions are effective to taper opioids in patients with chronic pain?' *BMJ* 2018;362:k2990 doi: 10.1136/bmj.k2990 (Published 27 September 2018).
- ['Top 10 tips: dependence forming medications'](#) produced by the RCGP succinctly outlines the key issues to consider when prescribing DFMs.
- 'Reducing opioid prescribing in chronic pain', PrescQIPP Feb-19.
- [NICE CG173, neuropathic pain in adults: pharmacological management in non-specialist settings](#).

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